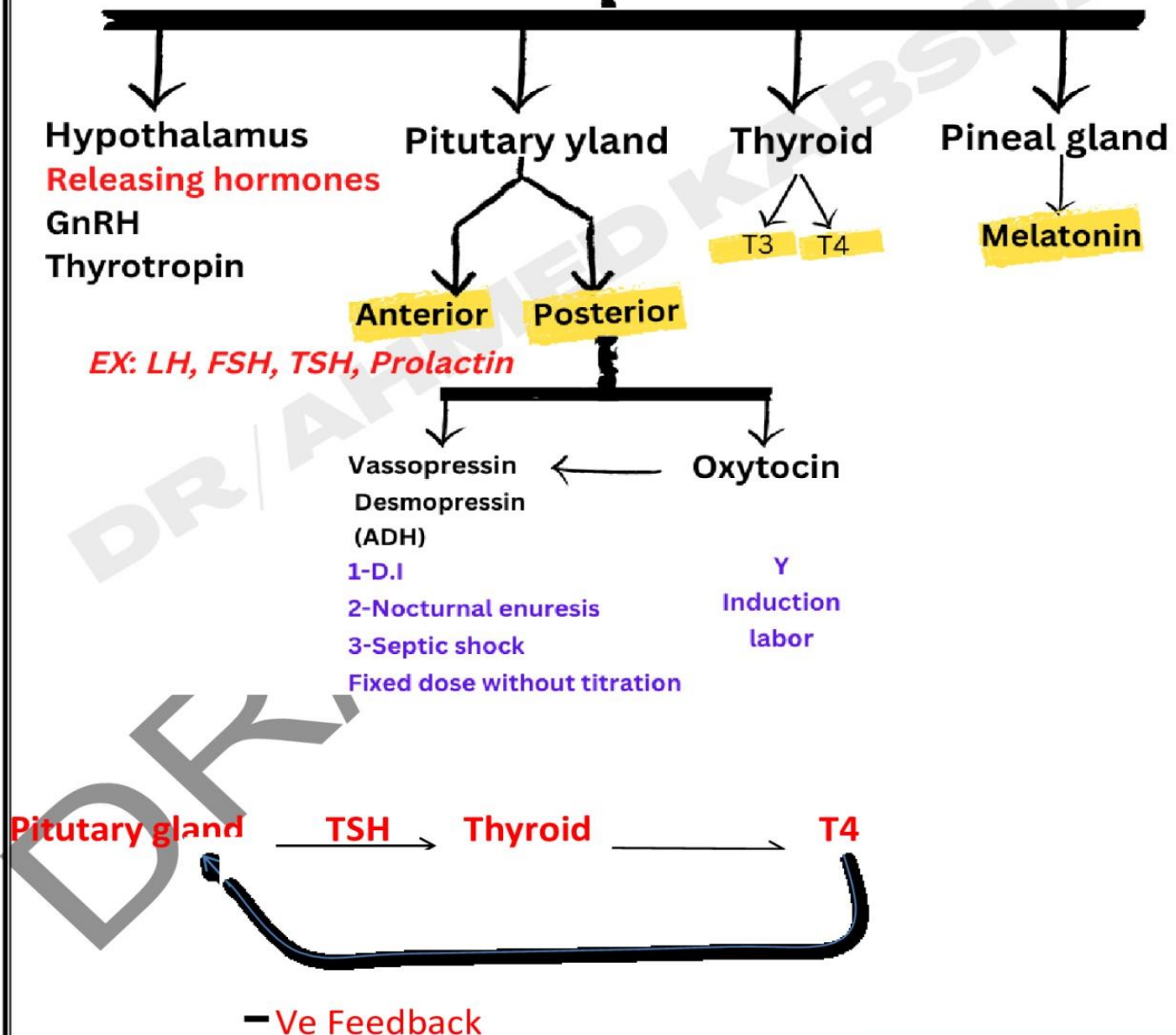
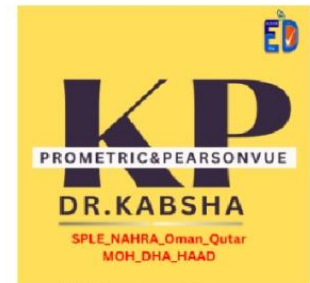




GLANDS



لا يسمح بتداول أي جهد خارج
المشاركين في الكورس وتعتبر أمانه



Shock

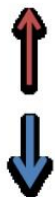
- **Septic shock** : 1st iv fluid
2nd NE or
fixed doses of vasopressin
Septic shock with kidney injury : dopamine
- **Cardiac shock** : dopamine
- **Anaphylactic shock**: epinephrine (adrenaline)

Vasopressin antidote : **Demeclocycline**, (a tetracycline antibiotic) or **phentolamin**

NE act as hormone & neurotransmitter

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❖ Thyroid:



← **Hyper**thyroidism (graves disease,, storm)

TSH

- **S/sx:** Heat intolerance, decrease weight, Goiter, Exophthalmos, tachycardia(propranolol)

Treatment

A. Antithyroid agents: Propylthiouracil (PTU) Methimazole

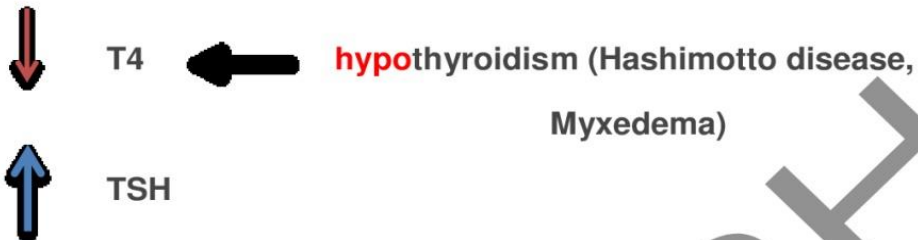
- In pregnancy {1st trimester PTU, 2nd & 3rd trimesters use methimazole} but safe in nursing mother.
- SE: Agranulocytosis (clozapine), PTU Hepatotoxic
- B. Iodides: Potassium iodide (KI), Saturated Solution of Potassium iodide (SSKI)
- Potassium perchlorate (NA/iodide symporter inhibitor) ttt AIT

❖ Crisis case of hyperthyroidism: Thyroid storm

- Life-threatening condition

➡ **Treatment:** PTU + SSKI + Dexamethasone + Propranolol (for symptoms) + Acetaminophen (for fever)

- Unlike antithyroid medications, radioactive iodine is a permanent and more reliable cure for hyperthyroidism. Radioactive iodine therapy can destroy all or part of the thyroid gland, depending on need

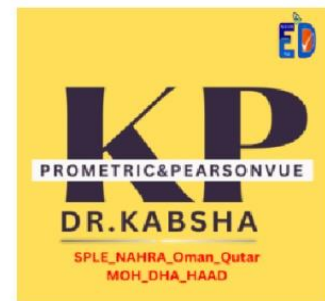


❖ **Diagnosis:**

Primary Hypothyroidism	low T4, High TSH
Secondary Hypothyroidism	low T4, Low TSH
Subclinical Hypothyroidism	Normal T4, High TSH

✚ S/sx: Cold intolerance, fatigue, increases weight.

Treatment



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Levothyroxine

Pregnancy:

.....)

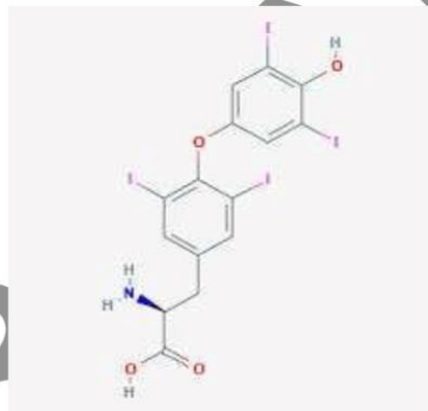
Increase dose by 30%-50%

HCG +Ve

Empty stomach???(

✚ **t_{1/2}: 7 day**

✚ 6-8 Weekes you will see the effect on patient energy & lab



Drug affect thyroid gland

✚ Amiodarone >>>>AIT....blue gray men skin
(photosensitivity)?????

✚ Lithium >>>>Monitor **renal** and **thyroid** every 6-12 months

✚ GLP-1: (Exenatide, Liraglutide) # Have adverse effect on **thyroid**

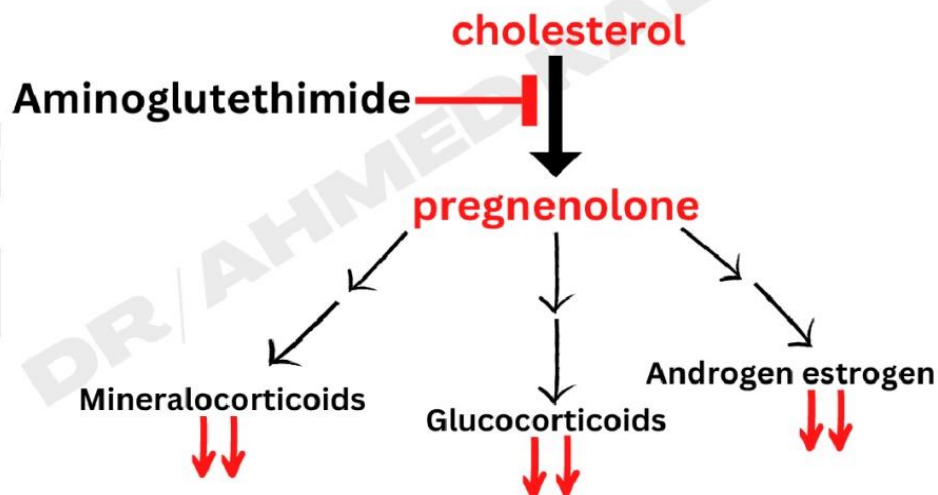
❖ Adrenal hormones:

Hyper Cortisone secretion:	Cushing syndrome
Hypo Cortisone secretion:	Addison's disease
Hyper Aldosterone secretion:	Conn's syndrome

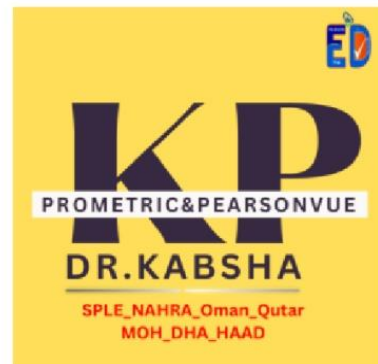
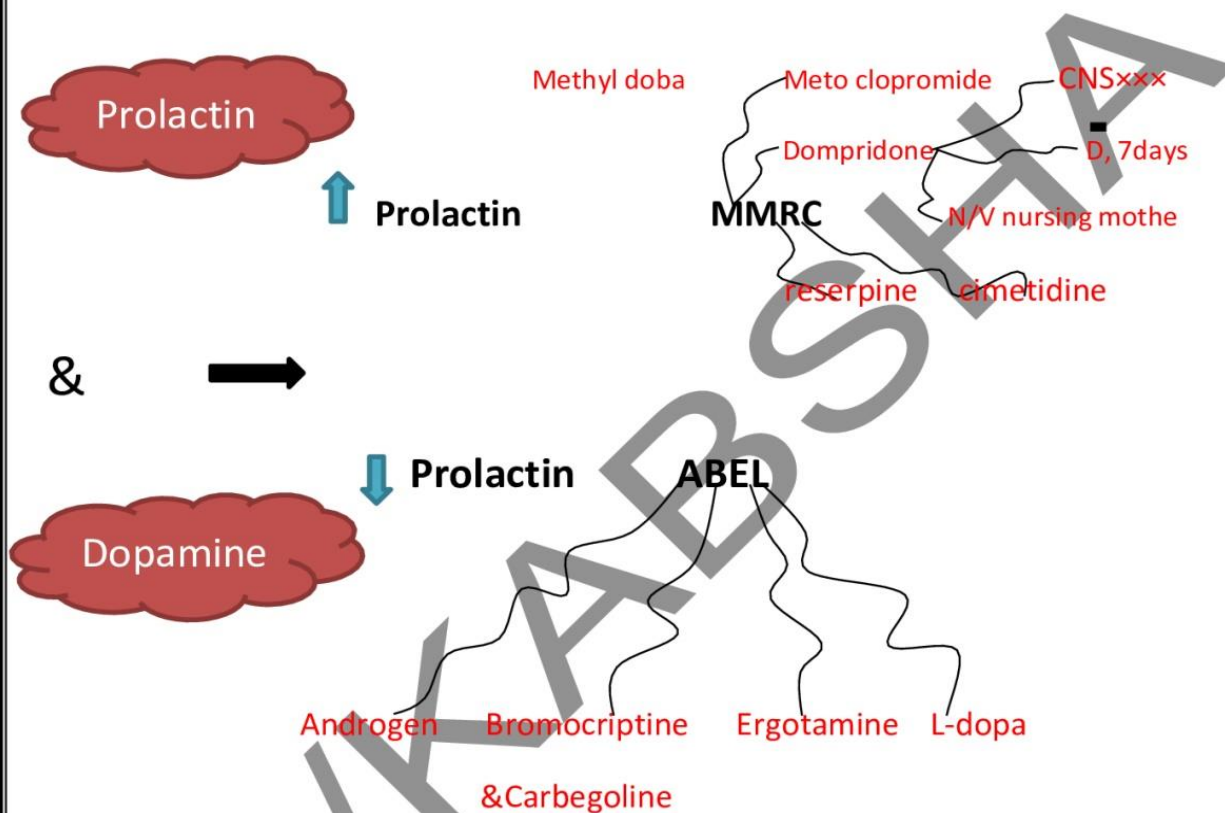
Cushing → increased cortisol (hypercortisolism) maybe increased adrenal gland size or over stimulation from pituitary Ttt

- 1- Aminoglutithimide antisteroid → conversion of cholesterol to pregnenolone → all steroids

*Inhibits the synthesis of **pregnenolone***



Ttt of cushing ? Addison ? conn??



→ Woman was taking a drug for migrain ,, she complained that her lactation was not sufficient ...what is medication she is taken for migraine ??

- a. propranolol.
- b. aspirin.
- c. ergotamine.
- d. acetaminophen .

❖ **Oral birth control** **progestin_n ± estrogen**

- For mid age woman want oral birth control to give: Ethinyl estradiol /lynestrenol
- EXCEPT if she has one of the following will give her: **LEVONORGESTREL**

§ > 35 + smoker or migraine headache or obese
§ > 50
§ Breastfeeding
§ DM + Vascular disease
§ Risk of DVT or Hx of
§ Hx of uncontrolled HTN or heart problems
§ Breast or endometrial cancer
§ Need to get pregnant

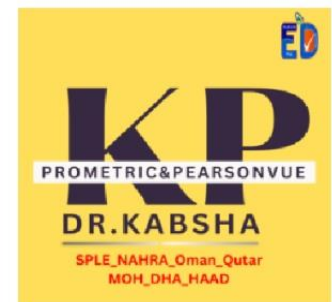
Emergency contraceptive ?? **a. hormonal** :

1.plan B ,,contraplan



2.mifepristone & ulipristal acetat (SPRM)

B . non hormonal : copper IUD



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Oral contraceptives suppress:	GnRH
Oral contraceptives work-on which phase:	in ovulation phase
Inhibit estrogen synthesis:	Tamoxifen in breast cancer
Monophase of oc:	fixed amount of progesterone and estrogen <i>ONLY</i>
Human chorionic gonadotropin is used to:	induce ovulation and treatment of infertility
Facial flushing and dry vaginal:	estrogen

abortion:

Substance that induce abortion: **Misoprostol**,
Mifepristone, **Methotrexate** Antiestrogen cause abortion:
Anastrozole

Antiprogesterone cause abortion: **Mifepristone**, **Misoprostol**

Prevention and treatment of postpartum and post
abortion hemorrhage: **Ergonovine**

Oral contraceptive used to treat: endometriosis-associate
pain "pain in menses and during intercourses"

infertility in women who do not ovulate: **Clomiphene citrate** — —



Post menopause

estrogen ± progestin

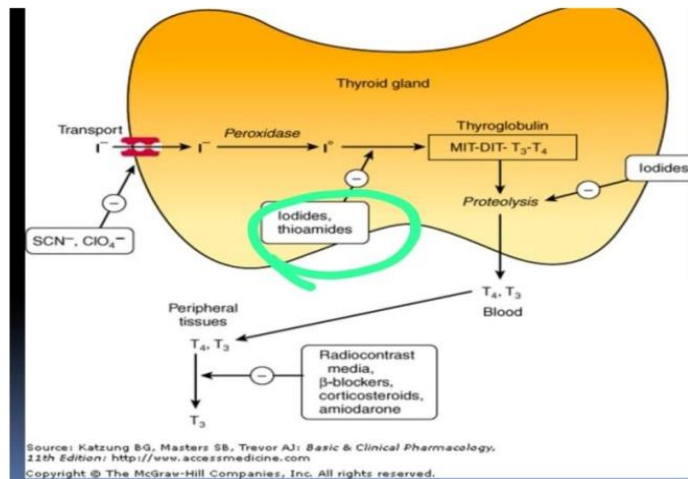
- Post menopause old woman is suffering from facial flushing & vaginal drying..She has done hysterectomy surgery....which drug should she use?
- a) progesterone
 - b) estrogen

Answer:

- If remove ovaries only ... need both estrogen & progesterone

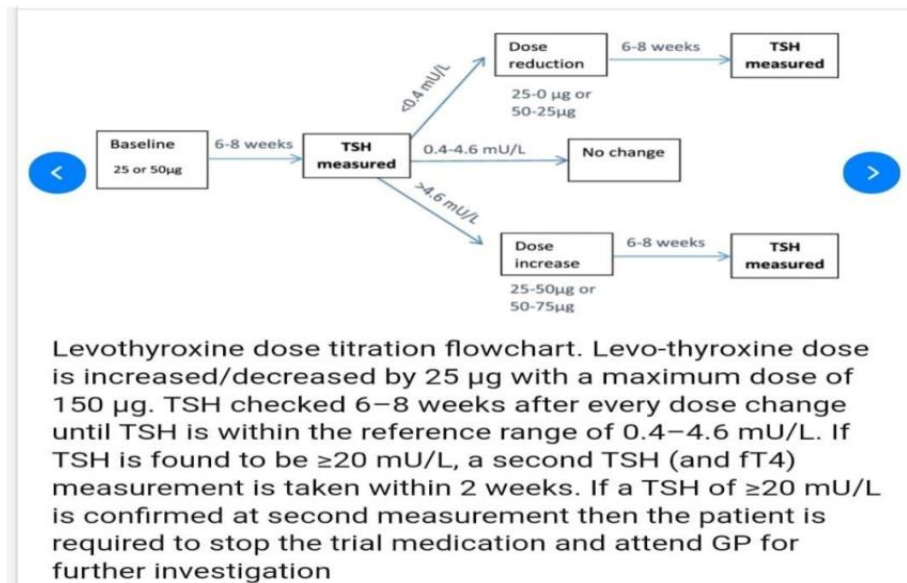
Estrogen alone <<< risk of uterus cancer

- if remove both uterus & ovaries...can use estrogen alone



How PTU help in treating hyperthyroidism:

- A. Decrease release of thyroid hormone
- B. Decrease iodine influx
- C. Inhibit thyroid peroxidase enzyme

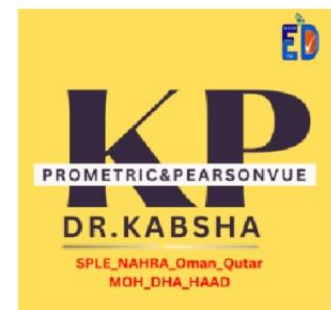


Patient is taking 50 mcg levothyroxine and TSH levels are still high, what adjustment should be made to treatment?

- A. Give dose of 100 mcg and check levels after 6-8 weeks
- B. Give dose of 150 mcg and check levels after 6-8 weeks
- C. Give dose of 25 mcg and check levels after 6-8 weeks
- D. Give dose of 75 mcg and check levels after 6-8 weeks

Prealbumin function?

- A. carrying the thyroid hormones
- B. carrying the oxygen
- C. carrying iron
- D. carrying blood



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